جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : امل بشرى رزق حنا

المحافظة : القليوبيه **الحي / المدينة :** العبور **العنوان :** الحى السابع

رقم البطاقة : 26907040101644 **رقم التليفون :** 01017693758 **تاريخ تسجيل الإستمارة :** 21-10-2026

العمر(تاريخ الميلاد) : 04-07-1969 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
الإخصائي الاجتماعي

التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة الإجابات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------

خطة العلاج : ----------

المستشفي : ----------**قسم :** ----------

جهة العلاج : ----------**نفقات :** ----------

ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: امل بشرى رزق حنا
National ID: 26907040101644

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bone scintigraphy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



امل
بشرى رزق حنا
ق ٨-بلوك ١٥٠٦٢-الحى السابع
العبور - القليوبية

٢٩٠٧٠٤٠١٠١٦٤٤

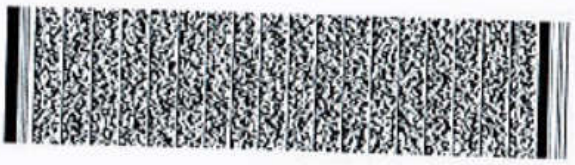
KX4289152



٢٩٠٧٠٤٠١٠١٦٤٤ ٢٠٢٤/١٢
حاصلة على دبلوم تجارة

أنثى مسيحية متزوجة
اشرف نبيه عزيز متى
البطاقة سارية حتى ٢٠٣١/١٢/١٣

١١



National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

MRN:	20225022529
Patient Name:	AMAL BOSHRA REZEK HANA
D.O.B:	04/07/1969
IC No.:	26907040101644
Sex:	Female
Location:	External Clinic

MOLECULAR GENETICS

Sish

Specimen Id	992509240038	**Collection Date**	24/09/2025 10:24 AM
Accession No	Z-SISH25-420	**Received Date**	24/09/2025 10:24 AM
Sample Type	Tissue	**Doctor**	Ashraf Nageb Mohamed Bayomy

Sish Report

pathology number

203311/25

Clinical Data

Lt. breast cancer

Gross Picture :

Referred block submitted for HER 2 detection by silver in situ hybridization.

System Used :

Ventana BenchMark ULTRA autostainer.

Probe:

Ventana Inform HER2 Dual ISH DNA Probe Cocktail

Scoring system :

Reference: ASCO–CAP HER2 Test Guideline Recommendations,2018 Twenty nuclei (each containing red (Chr17) and black (HER2) signals) should be enumerated. The final results for the HER2 status are reported based on the ratio formed by dividing the sum of HER2 signals for all 20 nuclei divided by the sum of Chromosome 17 signals for all 20 nuclei. The amplification status is defined as Amplified if the HER2/Chromosome 17 ratio >/=2.0 and the average Her2 gene copy number is >/=4.0. It is Non-Amplified if the HER2/Chromosome 17 ratio <2.0 with the Her2 gene copy number is <4.0.
If the HER2/Chr17 ratio is <2 and the Her2 gene copy number is between 4.0 and 6.0, or, HER2/Chr17 ratio is >/=2 and the Her2 gene copy number is <4, or HER2/Chr17 ratio is <2 and the Her2 gene copy number is >/= 6.0, an additional work should be done.

(Laterality) breast Cancer:

Left

Site 1

Diagnosis #1

Lt. breast Cancer:
Her-2 gene: amplified with clusters (Her2/Chr17 >2 and average Her2 copy number is >6).

Final Result #1

Amplified

Site 2

Co-Signed by:

MONA MOHAMED MAMDOUH

Reported and electronically endorsed by

Mona MOhamed Mamdouh Abdelfattah

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: امل بشرى رزق حنا **MR. :** 10784	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		**رقم طبي:**	

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

6/2026 MDT: 1/CONTINUE HERCEPTIN 2/PORT 3/ADJ AI(FEMARA +ZOLADEX +ZOMETA /6MS)

57 yr old female, postmenopausal >10 yrs ago, a case of left breast cancer cT2 N3 MO, received 4 AC 4TXT + herceptin outside (on priv basis) underwent CBS + ALND ypT2 ypN2a RCB 3 on 11/5/2026 MDT > continue herceptin + PORT + adj AI ECHO > pt will bring it for c1 her sc + femara + supp

for c2 herceptin sc, echo in 5/2026, new echo requested + femara BSM + APUS accepted for FU in 6 months pending PORT

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : هنا اسامه عبد الرحيم عبد البر

يعتمد ,

MRN:	20225022529
Patient Name:	AMAL BOSHRA REZEK HANA
D.O.B:	04/07/1969
IC No.:	26907040101644
Sex:	Female
Location:	External Clinic

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id 992508260050
Accession No Z-IMM25-203547
Sample Type Paraffin Block

Collection Date 26/08/2025 10:43 AM
Received Date 26/08/2025 10:44 AM
Doctor Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number
203311/25

Clinical Data
Left breast cancer

Gross Picture :
Referred block submitted for ER/PR immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:
Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :
According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:
- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.
- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:

left

Diagnosis :
ER: Positive, strong nuclear reaction in 90% of tumor cells [8/8].
PR: Positive, strong nuclear reaction in 85% of tumor cells [8/8].

Signature

MONA MOHAMED MAMDOUH
Reported and electronically endorsed by
Mona MOhamed Mamdouh Abdelfattah

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

MRN:	20225022529
Patient Name:	AMAL BOSHRA REZEK HANA
D.O.B:	04/07/1969
IC No.:	26907040101644
Sex:	Female
Location:	External Clinic

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id	992508260051	**Collection Date**	26/08/2025 10:43 AM
Accession No	Z-IMM25-203548	**Received Date**	26/08/2025 10:44 AM
Sample Type	Paraffin Block	**Doctor**	Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

203311/25

Clinical Data

Left breast cancer

Gross Picture :

Referred block submitted for HER 2 immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :

Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within <=10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within >10% of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in >10% of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in >10 % of tumor cells.

(Laterality) breast Cancer:

left

Her2 Result - 1

Equivocal [score 2+]. Recommended For SISH

Comment - 1

SISH will be done upon request

Signature

MONA MOHAMED MAMDOUH

Reported and electronically endorsed by

Mona MOhamed Mamdouh Abdelfattah

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

MRN:	20225022529
Patient Name:	AMAL BOSHRA REZEK HANA
D.O.B:	04/07/1969
IC No.:	26907040101644
Sex:	Female
Location:	External Clinic

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id	992508200137	**Collection Date**	20/08/2025 01:57 PM
Accession No	Z-SUR25-203311	**Received Date**	20/08/2025 01:57 PM
Sample Type	Tissue	**Doctor**	Abd ALLAH Mohamed Gamil

Clinical Information
Left breast (BIRADS 5) lesion, guided core biopsy.

Gross Picture
Multiple cores 0.8-1.5 cm.

Microscopic Picture
A case of invasive lobular carcinoma with focal ductal component, grade 2. There is associated DICS component, solid pattern, high grade. No evidence of lymphovascular emboli were detected. Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnostic Summary

Tumour type:
Invasive lobular carcinoma with focal ductal component, grade 2.

Laterality

Left

Specimen type:
Guided core biopsy

Location

Histological grade

Mitosis score (1-3):
2

Tubular formation score (1-3):
3

Nuclear grade (1-3):

2

Histological grade (1-3):
2

Diagnosis

Left breast lesion, guided core biopsy:
Invasive lobular carcinoma with focal ductal component, grade 2.

Comments
ER, PR & HER-2 are available (upon request).

Biopsy reporting category

B5b= Invasive cancer

Signature
Mona Alabrak, MD.

Co-signed by:

MRN:	20225022529
Patient Name:	AMAL BOSHRA REZEK HANA
D.O.B:	04/07/1969
IC No.:	26907040101644
Sex:	Female
Location:	External Clinic

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id	992508260050	**Collection Date**	26/08/2025 10:43 AM
Accession No	Z-IMM25-203547	**Received Date**	26/08/2025 10:44 AM
Sample Type	Paraffin Block	**Doctor**	Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

203311/25

Clinical Data

Left breast cancer

Gross Picture :

Referred block submitted for ER/PR immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :

According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

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Comment:

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- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:

left

Diagnosis :

ER: Positive, strong nuclear reaction in 90% of tumor cells [8/8].
PR: Positive, strong nuclear reaction in 85% of tumor cells [8/8].

Signature

MONA MOHAMED MAMDOUH

Reported and electronically endorsed by

Mona MOhamed Mamdouh Abdelfattah

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

MRN:	20225022529
Patient Name:	AMAL BOSHRA REZEK HANA
D.O.B:	04/07/1969
IC No.:	26907040101644
Sex:	Female
Location:	External Clinic

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id	992508260051	**Collection Date**	26/08/2025 10:43 AM
Accession No	Z-IMM25-203548	**Received Date**	26/08/2025 10:44 AM
Sample Type	Paraffin Block	**Doctor**	Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

203311/25

Clinical Data

Left breast cancer

Gross Picture :

Referred block submitted for HER 2 immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :

Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within <=10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within >10% of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in >10% of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in >10 % of tumor cells.

(Laterality) breast Cancer:

left

Her2 Result - 1

Equivocal [score 2+]. Recommended For SISH

Comment - 1

SISH will be done upon request

Signature

MONA MOHAMED MAMDOUH

Reported and electronically endorsed by

Mona MOhamed Mamdouh Abdelfattah

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

PATIENT NAME: AMAL BOSHRA REZEK HANA
GENDER: Female
DATE : 07-Oct-2025 03:39 PM

PATIENT ID: 20225022529
AGE: 56 y
ORDER: Z263333,Z263437

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC STUDY

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion.

FINDINGS:

1-CHEST

- Left breast central soft tissue mass lesion measures about 3 x 1.8 cm along its maximum axial diameters.
- Multiple left level I and II (sub pectoral) pathologically enlarged lymph nodes, the largest at level I measure about 1.8 x 1.2 cm along its maximum axial diameters.
- Prominent left supraclavicular lymph node, measures about 0.4 x 0.5 cm along its maximum axial diameters.
- Right upper lobe small calcified pulmonary nodules.
- Lingular and bilateral basal atelectatic bands with pleural tags.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Bulky right thyroid lobe with hypodense nodules, for US correlation.
- Bilateral elastofibroma dorsi more evident on the right side.

2- ABDOMEN AND PELVIS

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CT Scan
[[DOCTYPE:CT_SCAN]]

- The liver is of average size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation..
- Patent homogeneously enhancing portal vein and is main branches as well as its main tributaries.
- Cholycystectomy surgical clips.
- Normal appearance of the pancreas, spleen, kidneys,right adrenal, aorta, I.V.C and the pelvic organs.
- Bulky left adrenal gland.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.
- **The revised bone window revealed:**

1. No CT evidence of vertebral collapse or extra-osseous soft tissue component.
2. D4, D8 and L1 tiny sclerotic foci likely bone islands.
3. Mild bilateral sacroiliitis.

Opinion:

- **Left breast malignant looking mass lesion with ipsilateral pathological axillary lymph nodes.**
- **No CT evidence of definite pulmonary or hepatic metastatic deposits.**

Reported By,
Dr. Sara Refaat, MSc
Dr. Mennatallah Mohamed, MD **08-Oct-2025 12:41 PM**

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CT Scan
[[DOCTYPE:CT_SCAN]]

مركز بهية للاكتشاف المبكر
وعلاج سرطان السيدات

T
F
W
E

Patient Name:	**AMAL BOSHRA REZEK HANA**	Gender:	**Female**
MRN:	**20225022529**	Ordered By:	**Esraa mohamed ahmed mohamed hassan**
DOB:	**04/07/1969** Age: **56 Y 3 M 3 D**	Order ID:	**237780**

2D mammography

Procedure Date: 20/08/2025 09:52:00 AM

Report:

Report:
FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND
INDICATION:
- left breast mass, for assessment.
TECHNIQUE:
- CC and MLO view.
- Complementary ultrasound
REPORT:
LEFT BREAST:
- Scattered fibroglandular densities (ACR B).
- ill defined spiculated dense soft tissue mass seen at the UOQ about 5 cm from nipple, targeted ultrasound suspicious microcalcifications seen within the center of the mass.complementary ultrasound revealed ill defined non circumscribed mass. thin long spicules all seen radiating from its margin all around, the mass and its spicules measures 3.4x3.9 x2.8 cm in diameter.
- multiple level I pathological lymph nodes the largest measures 16x10 mm.
- Normal nipple areola complexes.
-
- RIGHT BREAST:
- Scattered fibroglandular densities (ACR B).
- No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.
OPINION:
- RIGHT BREAST: Normal study (BIRADS1).
- LEFT BREAST: unifocal malignant mass asociated with microcalcifications and level I pathological axillary lymph nodes (less than 5).BIRADS 5.

Approved by: Hanaa Abo Elmagd
Date:20/08/2025 Time:10:07

Impression:

Mammogram
[[DOCTYPE:MAMMOGRAM]]

PATIENT NAME: AMAL BOSHRA REZEK HANA
GENDER: Female
DATE : 20-Aug-2025 10:01 AM

PATIENT ID: 20225022529
AGE: 56 y
ORDER: Z237780,Z237781

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- left breast mass, for assessment.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

LEFT BREAST:

- Scattered fibroglandular densities (ACR B).
- ill defined spiculated dense soft tissue mass seen at the UOQ about 5 cm from nipple, targeted ultrasound suspicious microcalcifications seen within the center of the mass. complementary ultrasound revealed ill defined non circumscribed mass. thin long spicules all seen radiating from its margin all around, the mass and its spicules measures 3.4x3.9 x2.8 cm in diameter.
- multiple level I pathological lymph nodes the largest measures 16x10 mm.
- Normal nipple areola complexes.
-
- RIGHT BREAST:
- Scattered fibroglandular densities (ACR B).
- No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- RIGHT BREAST: Normal study (BIRADS1).
- LEFT BREAST: unifocal malignant mass asociated with microcalcifications and level I pathological axillary lymph nodes (less than 5).BIRADS 5.

RECOMMENDATION:

Referral to clinic

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مستشفى بهية الشيخ زايد
٤ الحي الأول - الشيخ زايد أمام كلية الهندسة

Mammogram
[[DOCTYPE:MAMMOGRAM]]

St. George's Clinic

54 Abd Elaziz Fahmy Street, Heliopolis

Telephone: 27756246 - 27756203

Name:Aml Boshra

Date: 8/14/2025

Digitalized low dose soft tissue Mammography with complementary breast US examination of both breasts report

- Mixed fibro-fatty glandular parenchyma.
- Left breast suspicious irregular opacity at UOQ suggesting of neoplastic lesion,no pathological micro calcification clusters.
- By US left breast shows irregular outline solid focal lesion at 1 o,clock measure more than1.9x1.8 cm with multiple foci of calcification.
- Normal central ducts system.
- Normal skin thickness and contour as well as nipple areola complexes.
- Left axillary tail myltiple LNS some are enlarged 1.8x0.93 with preserved fatty hilum

Opinion:

- **Left breast neoplastic lesionwith calcification WITH laxillary tail LNS,BIRADS5**

Best regards,

Dr. Nadia Abdalla Guindy

ADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

BIRADS 0	Needs further investigations
BIRADS 1	Normal findings
BIRADS 2	Benign
BIRADS 3	Probably benign
BIRADS 4	Probably malignant
BIRADS 5	Malignant
BIRADS 6	Proven pathologically malignant

Mammogram
[[DOCTYPE:MAMMOGRAM]]

St. GEORGE CLINIC

54 Abd – ElAziz Fahmy St. Heliopolis
Tel.: 26356246 – 26348203

Name	Mrs. Amal Boshra	Referred by	
Age	56 years	Date	9-10-2025

Measurements:

LVEDD	46	35– 57 mm	LA		40	25 – 40 mm
LVESD	27	25 – 40 mm	LAVi			≤35 ml/m²
IVSd	10	7 – 11 mm	RV	D1		≤41 mm
PWd	08	7 – 11 mm		D2		≤36 mm
FS	43	>25 %		D3		≤83 mm
EF	74	> 52 %	Ao		33	25-37 mm

Chambers:

- **Left ventricle:** Normal LV internal dimensions with normal wall thickness. Normal global systolic function. Estimated EF 74% by m-mode and 2 D method
- No evidence of RSWMA
- Normal LV diastolic function
- **Left atrium:** Upper normal dimension
- **Right atrium:** Normal RAA
- **Right ventricle:** Normal dimension, normal systolic function

Valves:

- **Mitral valve:** Normal leaflet thickness with adequate excursion. No mitral stenosis. Mild mitral incompetence.
- **Aortic valve:** Trileaflet, sclerotic leaflets with adequate excursion. Trivial aortic incompetence. No significant gradient across.
- **Tricuspid valve:** Normal leaflet thickness with adequate excursion. Trivial tricuspid incompetence. ESPAP 22mmHg
- **Pulmonary valve:** Normal leaflets morphology, motion and flow.
- Normal aortic root dimension
- Normal pericardium with no pericardial effusion
- No intracardiac masses or thrombi by TTE
- Intact IAS and IVS by TTE

Impression:

- Normal LV dimensions and systolic function. LVEF 74%
- No evidence of RSWMA
- No significant valvular heart disease

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Pathology Report
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